



Final Pathologic Diagnosis:

A. Ureter, left distal, biopsy for frozen section diagnosis:

Negative for carcinoma.

Frozen section diagnosis is confirmed.

B. Ureter, right distal, biopsy for frozen section diagnosis:

Negative for carcinoma.

Frozen section diagnosis is confirmed.

C. Urinary bladder, cystoprostatectomy:

Tumor histologic type: Urothelial carcinoma

Variant histology (%): None

Tumor size: greatest dimension: 6.5 cm

Additional dimensions: 3 x 2.5 cm

Grade (WHO): High

Tumor configuration: Papillary and nodular/infiltrative

Associated epithelial lesions: Urothelial carcinoma in situ

Microscopic tumor extension: Tumor invades the peri-vesicle soft tissue

Lymph-vascular invasion: Yes

Margins: Negative

Urethral: Negative

Ureteral: Negative

Paravesical soft tissue: Negative

Other (specify): Periprostatic soft tissue

Additional pathologic findings: Extensive perineural invasion

Paravesical lymph nodes: None

All lymph nodes: 1/15 (see parts D, E, F, G)

Size of largest lymph node metastasis: 10 mm

Extranodal extension (location): No

*ICD-0-3
carcinoma, urothelial, NOS 8120/3
Site: bladder, NOS C67.9*

*hw
2/5/12*

Pathologic stage (): pT4a pN1 pM-Not applicable

Prostate, cystoprostatectomy:

Urothelial carcinoma involving prostatic urethra and invading prostatic stroma (slide C13 and C19).

Extensive perineural (slide C11, C12 etc) and lymphovascular invasion (C22)

D. Lymph nodes, left inferior pelvic, excision:

One out of two lymph nodes negative for metastatic carcinoma (1/2).

Dimension of largest lymph node metastasis, 10 mm (slide D2).

E. Lymph node, left superior pelvic, biopsy:

One lymph node negative for metastatic carcinoma (0/1).

F. Lymph nodes, right inferior pelvic, excision:

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
ICD/A Discrepancy	☒	☒
Prior Malignancy History	☒	☒
Dual/Synchronous Primary Node	☒	☒
Case Is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	<i>hw</i>	<i>hw</i>
Date Reviewed:	<i>2/5/12</i>	

Eight lymph nodes negative for metastatic carcinoma (0/8).

G. Lymph nodes, right superior pelvic, excision:

Four lymph nodes negative for metastatic carcinoma (0/4).

H. Appendix, appendectomy:

No pathologic change.

The examination of this case material and the preparation of this report were performed by the staff pathologist.

Intraoperative Consult Diagnosis:

FSA: Ureter, left (stitch distal):

Negative for carcinoma.

F/S TAT: 11 mins.

FSB: Ureter, right (stitch distal):

Negative for carcinoma.

F/S TAT: 10 mins.

Gross Description:

Received are eight containers each labeled with the patient name.

Part A is received fresh for frozen section diagnosis and is additionally labeled "left ureter stitch distal." The specimen consists of a segment of ureter having a length of 0.3 cm and a diameter of 0.5 cm. The residual frozen section tissue is entirely submitted in cassette A for permanent sections.

Part B is received fresh for frozen section diagnosis and is additionally labeled "right ureter stitch distal." The specimen consists of a segment of ureter having a length of 0.5 cm and a diameter of 0.8 cm. The specimen is bisected at the time of frozen section diagnosis. The residual frozen section tissue is entirely submitted in cassette B for permanent sections.

Part C is received fresh for _____ and is additionally labeled "bladder and prostate." The specimen consists of a cystoprostatectomy specimen, having overall dimensions of 8.5 x 8 x 7.5 cm. The specimen consists of a 4.5 x 4 x 3.5 cm bladder with attached bilateral ureters (right ureter length 4 cm and left ureter length 3 cm) and a 4.5 x 3.5 x 3.5 cm prostate with attached bilateral seminal vesicles and vas deferens. The prostate is inked as follows: right side yellow, left side black, and posterior red. Anteriorly, the bladder is inked black. The urethral margin is shaved. The specimen is opened anteriorly to reveal a 6.5 x 3 x 2.5 cm partially exophytic and partially ulcerated pink-tan mass involving the inferior half of the bladder circumferentially. The dome of the bladder appears uninvolved. The mass appears to invade into the muscularis propria and comes to within 0.2 cm of the

inked anterior (closest) soft tissue margin. Additionally, the mass appears to invade the prostate and comes to within 1.2 cm of the urethral margin. The uninvolved bladder mucosa is pink-tan, wrinkled and somewhat edematous. After amputating the prostate from the bladder and removing the bilateral seminal vesicles and vas deferens, the prostate weighs 49 g. There is a moderate amount of perivesicular fat, and after the use of a chemical clearing agent, one tan lymph node is identified, having dimensions of 0.8 x 0.4 x 0.2 cm.

Representative sections are submitted as follows:

- C1 urethral margin;
- C2 uninvolved bladder mucosa;
- C3 mass to nearest soft tissue margin (anterior);
- C4-6 mass;
- C7 mass to uninvolved;
- C8-13 right posterior prostate, submitted from apex to base;
- C14-19 left posterior prostate, submitted from apex to base;
- C20-21 right anterior prostate, submitted from apex to base (from mid portion of prostate gland);
- C22,23 left anterior prostate, submitted from apex to base (from mid portion of prostate gland);
- C24 right seminal vesicle;
- C25 left seminal vesicle;
- C26 one whole lymph node.

Part D is received in formalin and is additionally labeled "left inferior pelvic lymph nodes." The specimen consists of a single portion of fibroadipose tissue, having dimensions 7 x 2.5 x 1.2 cm. After the use of a chemical clearing agent, two tan lymph nodes are identified, having dimensions 3 x 1.6 x 0.8 cm and 0.8 x 0.6 x 0.2 cm. Representative sections are submitted as follows:

- D1-2 one lymph node bisected;
- D3 one whole lymph node.

Part E is received in formalin and is additionally labeled "left superior pelvic lymph node." The specimen consists of a single portion of fibroadipose tissue, having dimensions 2.8 x 1 x 0.8 cm. After the use of a chemical clearing agent, a single tan lymph node is identified, having a greatest dimension of 1.5 cm. The specimen is bisected and entirely submitted in cassettes E1-E2 (one lymph node bisected).

Part F is received in formalin and is additionally labeled "left inferior pelvic lymph nodes." The specimen consists of a single portion of fibroadipose tissue, having dimensions 4.5 x 3 x 1.5 cm. After the use of a chemical clearing agent, two tan lymph nodes are identified, having greatest dimensions of 3 and 2.8 cm. Representative sections are submitted as follows:

- F1-3 one lymph node trisected;
- F4-6 one lymph node trisected.

Part G is received in formalin and is additionally labeled "right superior pelvic lymph nodes." The specimen consists of a single portion of fibroadipose tissue, having dimensions 2.5 x 1.5 x 0.8 cm. After the use of a chemical clearing agent, three lymph nodes are identified, with greatest dimensions ranging from 0.5 to 1.2 cm. Representative sections are submitted as follows:

- G1 two whole lymph nodes;
- G2 one lymph node bisected.

Part H is received in formalin and is additionally labeled "appendix." The specimen consists of an appendix having a length of 7 cm and an external diameter ranging from 0.4 cm distally to 0.6 cm proximally. The proximal surgical margin is received opened. The serosal surface of the appendix is pink-tan, smooth and glistening. There is no evidence of perforation or exudate formation. The attached mesoappendix has dimensions 2 x 0.5 x 0.3 cm. The appendiceal wall thickness is 0.2 cm throughout. The lumen contains scant fecal material. A fecalith is not present. The mucosa is tan. No masses are identified. The inter luminal diameter ranges from 0.3 cm to pinpoint. Standard sections are submitted in cassette H.

Microscopic Description:

The final diagnosis of each specimen incorporates the microscopic examination findings.

END OF REPORT
